

Overview

Suicide in children and adolescents is a major public health issue. Nationally, suicide is the second leading cause of death for 10- to 19-year-olds. Suicide affects people of all backgrounds. No one is immune. However, there are ways to decrease risk and help young people navigate crises. Most children and adolescents who attempt suicide show warning signs. It is vital that families are aware of these signs and discuss concerns, ideally prior to a life-threatening mental health crisis.

- According to the Youth Risk Behavior Survey in 2023, just over 1 in 5 high school students reported seriously considering suicide in the past year. Additionally, 1 in 11 high school students indicated they had made a suicide attempt within the past year.

Research is helping us better understand youth suicide.

- Suicide attempts are far more common than deaths by suicide, but any suicidal behavior is serious and can have lasting consequences.
- Most teens who are suicidal do not actually wish to die, but they are in severe emotional pain and want the pain to stop. They may not know how to move forward and are unsure if there are ways to feel better.

The good news is that suicides are largely preventable. Early identification, timely treatment and support from families, schools, friends and communities play key roles.

What Are Suicidal Behaviors?

- By definition, a **suicide** is a death caused by self-directed, harmful behavior with an intent to die.
- A **suicide attempt** is a non-fatal, self-directed, potentially harmful behavior with an intent to die.

- **Suicidal ideation** refers to thinking about, considering or planning suicide.

Although the prevalence of mental health disorders and rates of suicide increase as children grow older, even young children express thoughts of suicide and have taken their own lives. Recent research suggests that about 5% of 9 and 10-year-olds have experienced thoughts of suicide.

There are well established differences between boys and girls in suicide rates. Girls are two to three times more likely to express thoughts of suicide and attempt suicide. However, boys are four times more likely to die by suicide than girls. This is because boys tend to use more lethal suicide attempt methods and take action more impulsively.

Suicide is rare in children before twelve years of age but can occur. Family and school problems are notable risk factors for suicide in young children. Warning signs should always be taken seriously. Because young children can move from thoughts of suicide to an attempt quickly and without planning, universal precautions should be taken around lethal means.

Signs & Symptoms

What Warning Signs Should Parents Look For?

Family members, caring adults and friends should respond whenever a child talks about or posts on social media about wanting to die or hurt themselves on purpose. Children expressing such ideas are typically experiencing emotional distress and may not know the skills or tools to use to help them through these stressful experiences. Mental health is as important as physical health and requires a lot of support and a large safety net.

The following warning signs indicate that a youth is severely depressed or at risk for suicide and should be responded to **immediately**:

- Talking to others or posting on social media about:
 - Suicide or wanting to die
 - Feeling hopeless, trapped or like they are “a burden” to others
- Looking for ways to kill themselves:
 - Gathering medications, sharp objects or firearms
 - Searching online for ways to end their life

- Expressing unbearable emotional pain
- Visiting or calling people to say “goodbye”
- Giving away prized possessions
- Suddenly becoming calm or cheerful after a long period of depression

The following warning signs indicate that a youth may be struggling with depression, which requires evaluation by a mental health professional:

- Feeling sad or irritable more often than not
- Sleeping or eating more or less than usual
- Showing little to no interest in pleasurable activities
- Withdrawing from others
- Participating in reckless behavior that is out of character
- Engaging in self-injurious behavior
- Having trouble concentrating or performing poorly in school
- Complaining frequently about physical symptoms (e.g., fatigue, stomachaches, headaches)

- Increasing use of alcohol and drugs

Risk Factors

What Are the Risk Factors for Suicide in Children?

Suicide is complex and can rarely ever be attributed to one single cause.

Risk factors for suicide include internal and environmental factors. Although some risk factors can be changed and others cannot, making families aware of the risks is important. Suicide risk is greatest when a youth has multiple risk factors.

Risk factors may include:

- Previous suicide attempt
- Mental health disorders, especially depression and other mood disorders, anxiety disorders
- Substance abuse
- History of abuse or neglect

- Family history of mental health disorders and/or suicide
- Feelings of hopelessness
- Impulsivity
- Access to lethal means for suicide (e.g., firearms)
- Exposure to others who have died by suicide
- Distress related to gender identity or sexual orientation
- Exposure to bullying or bullying others

What Are Protective Factors Against Suicide?

Protective factors are strengths that protect youth from attempting or dying by suicide. It is important to increase the availability of protective factors to at-risk youth.

- Feeling supported by family
- Reasons to live and future goals
- Social or academic support

- Therapeutic relationship with a therapist
- Strong coping and problem-solving skills
- Family can eliminate access to lethal means during crisis

Treatment

How Are Suicidal Thoughts and Behaviors Treated?

The level of intervention for suicidal thoughts and behaviors depends on the level of suicide risk. In Franklin County, it is strongly recommended that families call the Youth Psychiatric Crisis Line at **614-722-1800 (tel:614-722-1800)** so professionals can support the evaluation of risk and level of care required. Often crisis clinicians can support a plan to increase safety and begin the process of getting effective mental health care. When there is an imminent risk of suicide, immediate psychiatric evaluation and/or hospitalization may be recommended. If medical attention is needed related to a suicide attempt or self-injury, parents should take children directly to the closest emergency room.

If the risk is not imminent and safety of the child is clearly established, the child may be referred to a mental health professional for outpatient treatment.

Two interventions that have proved effective in treating youth at risk for suicide in the outpatient setting are:

- Cognitive behavior therapy (CBT)
- Dialectical behavioral therapy (DBT)

Regardless of therapy type, clinicians supporting the mental health care of a child with a history of suicidal thoughts and/or behaviors should be comfortable directly treating suicidal behaviors as well as underlying mental health conditions. It is also important that treatment be highly collaborative, and the family is involved throughout the course of treatment.

Seeking Help

When Should You Seek Help?

When a youth shares thoughts of wanting to die or displays warning signs, consider the following strategies:

- Remain calm and take a deep breath. It's OK to feel uncomfortable.
- Be patient and speak in a relaxed, reassuring tone.
- Tell your child you care and acknowledge they are hurting.
- Be direct about your concerns:

- State the specific changes you see in your child's mood and/or behavior.
- Ask if they are thinking about suicide or have tried to kill themselves.
- Get professional help.

Some ways of responding to a youth who is having suicidal thoughts are ineffective. Please consider the following tips:

- Don't debate whether suicide is right or wrong.
- Don't lecture on the value of life or question why someone could feel this way. You do not have to agree with how they feel to offer support.
- Don't be sworn to secrecy or promise confidentiality. Share that safety and getting help are top priority.
- Convey the message that suicide is preventable and treatment is effective.

Always treat suicidal threats or behaviors as an emergency. If your child reports suicidal thoughts, threatens suicide or shows suicidal behaviors, do not leave them alone. All disclosures should be taken seriously every time no matter the child's age.

Call 9-1-1 or your local emergency number immediately if your child has made a suicide attempt or is showing warning signs of suicide, or take your child to the nearest emergency room.

In Franklin County, call (614) 722-1800 (tel:(614) 722-1800) for the Franklin County Youth Psychiatric Crisis Line for youth ages 17 and younger. You can also call, text, or chat using the Suicide & Crisis Lifeline at 988.

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The information provided here is only for general reference and should not take the place of medical care or patient education. If you have any questions, please call your child's care team.

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